

140mm.

LIOBAC TABLET 10 MG**CONTENT**

Each plain tablet contains Baclofen 10 mg.

PRODUCT DESCRIPTION

White round tablet embossed with "SPS", "L05" and a breakline on one side and plain on the other.

PROPERTIES**Pharmacodynamics**

Baclofen, an analogue of gamma-aminobutyric acid (GABA), is a centrally acting skeletal muscle relaxant. It inhibits monosynaptic and polysynaptic transmission at the spinal cord level by stimulating the GABA_A receptors, resulting in inhibition of the release of the excitatory amino acids glutamate and aspartate. Baclofen may act at supraspinal sites producing CNS depression but has no effect on neuromuscular impulse transmission. Baclofen also exerts an antinociceptive effect but the clinical significance of this is unknown.

Pharmacokinetics

Absorption: Baclofen is rapidly and completely absorbed from the gastrointestinal tract following an oral dose. The peak plasma concentration occurs in 0.5 to 3 hours following ingestion, but the rate and extent of absorption vary between patients, and vary inversely with the dose.

Distribution: The distribution volume of baclofen is 0.7 L/kg and approximately 30% of baclofen is bound to plasma protein. In cerebrospinal fluid, the active substance concentrations are approximately 8.5 times lower than in the plasma.

Metabolism: About 15% is metabolized in the liver to the pharmacologically inactive, β-[p-chlorophenyl]-4-hydroxybutyric acid.

Elimination: The plasma elimination half-life of baclofen is about 3–4 hours. About 75% of the dose is excreted via the kidney within 72 hours, mainly as unchanged drug. The remainder of the dose, including 5% in the form of metabolites, is excreted in faeces.

INDICATION

Liobac is indicated for the alleviation of spasticity of the skeletal muscles in multiple sclerosis and other spastic conditions occurring from spinal cord diseases of infectious, degenerative, traumatic, neoplastic or unknown origin (e.g. spastic spinal paralysis, amyotrophic lateral sclerosis, syringomyelia, transverse myelitis, traumatic paraplegia or paraparesis, and compression of the spinal cord), muscle spasticity of cerebral origin, especially where due to infantile cerebral palsy.

Because of the likelihood of intolerance, **Liobac** is less suitable for spasticity due to cerebrovascular accident or degenerative or neoplastic brain disease, but it may be tried if administered cautiously.

RECOMMENDED DOSAGE

Liobac treatment should always be initiated with small doses which are then gradually increased. The optimum daily doses should be individually adapted to each patient's requirement so that clonus, flexor and extensor spasm and spasticity are reduced and possible adverse effects are avoided.

In order to prevent excessive weakness and falling, **Liobac** should be used with caution when spasticity is needed to maintain upright posture and balance in locomotion or other functions. It may be important to maintain a certain degree of muscle tone and allow occasional spasms to help support circulatory function and, possibly, prevent deep vein thrombosis.

Liobac should be taken during meals with a little liquid. The daily dosage should be given in divided doses preferably 3 in adults and 4 in children.

Abrupt discontinuation of treatment should be avoided. If the therapeutic effect is not apparent within 6–8 weeks of achieving the maximum dosage, a decision whether to continue with **Liobac** should be taken.

Adults: Treatment should be started at doses of 5 mg three times a day for 3 days. Then, each dose should subsequently be increased by 5 mg every 3 days until the desired response is reached. In certain patients reacting sensitively to the drugs, it may be advisable to begin with lower daily dosage (5 mg or 10 mg), and then increased by smaller steps at longer intervals. The optimum dosage generally ranges from 30 to 80 mg daily although daily doses of 100 to 120 mg may be given to carefully supervised patients in hospitals.

Children: Experience with baclofen tablets is limited in children and adolescents. The minimum age for treatment is 12 months. Treatment should be started at very low dose (0.3 mg/kg/day in 4 divided doses) and should be cautiously raised at intervals of about 1–2 weeks until it becomes sufficient for the individual patient's requirements.

The recommended daily maintenance doses are as follow:

- 10 years of age and below: 0.75–2 mg/kg
- Over 10 years of age: Up to 2.5 mg/kg

Dose should not exceed maximum dose of 40 mg/day in children below 8 years of age and 60 mg/day in children over 8 years of age.

Use in patients with renal impairment

Lower doses (approximately 5 mg daily) should be used with caution for patients with impaired renal function or patients undergoing chronic haemodialysis. Baclofen should not be used in patients with end-stage renal failure unless the potential benefit outweighs risk of further treatment. These patients should be monitored for early signs and symptoms of overdosage (e.g. drowsiness, lethargy).

Elderly patients and patients with spasticity of cerebral origin

Dosage should be increased particularly slowly in elderly and weak patients suffering from organic brain disorders, cardiovascular disease, respiratory failure, or hepatic or renal impairment, and also in patients with spasticity of cerebral origin. Elderly patients should be kept under appropriate surveillance. Toxicity due to baclofen may be mistaken for uremic encephalopathy.

CONTRAINDICATIONS

Liobac is contraindicated in patients with:

- 1) Known hypersensitivity to baclofen or to any of the excipients
- 2) Active peptic ulcer disease

WARNINGS AND PRECAUTIONS**Psychotic disorders and epilepsy**

Patients suffering from psychotic disorders, schizophrenia, depressive or manic disorders, confusional states or Parkinson's disease should be treated cautiously and closely monitored as exacerbations of these disorders may occur.

Baclofen may be used under appropriate supervision in patients with epilepsy and muscle spasticity (provided adequate anticonvulsive therapy is continued) as the seizure threshold may be reduced. There have also been occasional reports of seizures associated with cessation of baclofen therapy or with overdose.

Renal impairment

Baclofen should be used with caution in patients with renal impairment and should be administered to end-stage renal failure patients only if the expected benefit outweighs the potential risk (see *Recommended Dosage*).

Caution is also required when baclofen is combined with drugs that can significantly impact renal function. Renal function should closely monitored and baclofen daily dosage adjusted accordingly to prevent baclofen toxicity.

Peptic ulcer diseases

Baclofen stimulated gastric acid secretion and should be used with caution in patients with a history of peptic ulcer and avoided in those with active peptic ulcer disease.

Urinary disorders

Neurogenic disturbances, affecting emptying of the bladder may show an improvement during treatment with baclofen. However, acute retention of urine may occur in patients with pre-existing sphincter hypertonia. Hence, baclofen should be used with caution in these circumstances.

Others

Liver function should be monitored in patients with liver disease. Baclofen should also be used with caution in patients with respiratory impairment.

Laboratory tests

There have been rare reports on elevated serum glutamic-oxaloacetic transaminase (SGOT), alkaline phosphatase and glucose levels. Hence, appropriate laboratory tests should be performed periodically in patients with liver diseases or diabetes mellitus in order to ensure that no drug-induced changes in these underlying diseases have occurred.

Abrupt withdrawal:

Following abrupt withdrawal of baclofen, anxiety and confusional state, delirium, hallucination, psychotic disorder, mania or paranoid, convulsion (status epilepticus), dyskinesia, tachycardia, hyperthermia and - as a rebound phenomenon - temporary aggravation of spasticity have been reported especially after long-term medication. Therefore, except in emergencies or where serious adverse effects have occurred, the dose of baclofen should be reduced gradually over a period of approximately 1 to 2 weeks (longer if symptoms occur).

Effects on ability to drive or operate machinery

Baclofen may cause sedation and decreased alertness and hence may adversely affect the patient's ability to react. Therefore, patient should exercise due caution when driving a vehicle or operating machinery.

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PREGNANCY AND LACTATION

Pregnancy: There are no adequate and well-controlled studies in pregnant women. As baclofen crosses the placenta barrier, the drugs should not be used during pregnancy unless the potential benefits outweigh the unknown potential hazards to the foetus.

Lactation: In mothers taking baclofen in therapeutic doses, the active substance passes into the breast milk, but in quantities so small that no undesirable effects on the infant are to be expected.

DRUG INTERACTIONS

Concomitant Drugs	Drug Interactions
CNS agents, alcohol, or synthetic opiates	Exacerbate CNS effects of baclofen and should be avoided. Concomitant use of CNS depressants may potentiate impaired cognitive function and motor skills and increase the risk of respiratory depression, hypotension, oversedation and syncope. If CNS depressant have to be used concurrently, consider dose reduction of CNS depressant and use with caution. The risk of respiratory depression is also increased. Careful monitoring of respiratory and cardiovascular functions is essential, especially in patients with cardiopulmonary disease and respiratory muscle weakness.
Antihypertensive	The dosage of antihypertensive drugs should be adjusted accordingly since the combined treatment of these drugs and baclofen is likely to increase the fall in blood pressure.
Antidepressants	May potentiate the effects of baclofen and result in pronounced muscular hypotonia. Antidepressants, specifically imipramine, amitriptyline and clomipramine induce short-term memory loss.
Lithium	Concomitant use of oral baclofen and lithium resulted in aggravated hyperkinetic symptoms.
Drugs used in parkinsonism	Several reports of mental confusion, hallucination, headache, nausea and agitation have been reported in patients with Parkinson's disease receiving baclofen and levodopa (alone or in combination with Dopa-decarboxylase (DDC) inhibitor, carbidopa).
Ibuprofen and other drugs that produce renal insufficiency	May reduce baclofen excretion leading to toxicity.

ADVERSE REACTIONS

Adverse effects occur mainly at the start of treatment if the dosage is increased too rapidly or if large doses are used. These effects are often transitory and can be attenuated or eliminated by reducing the dosage and they are seldom severe enough to necessitate withdrawal of the medication.

Patients with case history of psychiatric illness or with cerebrovascular disorders (e.g. stroke) and elderly patients are inclined to suffer more serious adverse reactions. Lowering of the convulsion threshold and convulsions may occur, particularly in epileptic patients. Certain patients have shown increased muscle spasticity as a paradoxical reaction to the medication. Many of the side effects reported are known to occur in association with the underlying conditions being treated.

Nervous system disorders

Very common: Sedation, somnolence
Common: Respiratory depression, confusional state, dizziness, hallucination, depression, fatigue, insomnia, euphoric mood, muscular weakness, ataxia, tremor, nightmare, myalgia, headache, nystagmus, dry mouth
Rare: Paraesthesia, dysarthria, dysgeusia

Eye disorders

Common: Visual impairment, accommodation disorders

Cardiac disorders

Common: Decreased cardiac output
Not known: Bradycardia

Vascular disorders

Common: Hypotension

Gastrointestinal disorders

Very common: Nausea
Common: Gastrointestinal disorder, constipation, diarrhoea, retching, vomiting

Rare: Abdominal pain

Hepatobiliary disorders

Rare: Abnormal hepatic function

Skin and subcutaneous disorders

Common: Rash, hyperhidrosis
Not known: Urticaria

Renal and urinary disorders

Common: Pollakiuria, enuresis, dysuria
Rare: Urinary retention

Reproductive system and breast disorders

Rare: Erectile dysfunction

General disorders and administration site conditions

Very rare: Hypothermia
Not known: Drug withdrawal syndrome (see Warnings and Precautions)

Investigations

Not known: Increased blood glucose

OVERDOSAGE

Symptoms: Symptoms of overdose are predominantly those of central nervous system depression which include drowsiness, impairment of consciousness, respiratory depression and coma. Other symptoms that are likely to occur are confusion, agitation, hallucination, convulsion, accommodation disorder, absent papillary reflex, generalized muscular hypotonia, myoclonia, hyporeflexia or areflexia, peripheral vasodilation, hypotension, hypothermia, bradycardia, nausea, vomiting, diarrhoea, hypersalivation and elevated liver enzymes. A deterioration of the overdose syndrome may occur if various substances or drugs acting on the central nervous system (e.g. alcohol, diazepam, tricyclic antidepressants) have been taken at the same time.

Treatment: No specific antidote is known. Supportive measures and symptomatic treatment should be given for complications such as hypotension, hypertension, convulsions, gastrointestinal disturbances, and respiratory or cardiovascular depression. After ingestion of a potentially toxic amount, activated charcoal should be considered, especially during the early period after ingestion. Gastric decontamination (e.g. gastric lavage) should be considered in individual cases, especially in the early period (60 minutes) after ingestion of a potentially life-threatening overdose. Comatose or convulsing patients should be intubated prior to the initiation of gastric decontamination. Since the drug is excreted chiefly via the kidneys, generous quantities of fluid should be given, possibly together with a diuretic. Haemodialysis (sometimes unscheduled) may be useful in severe poisoning associated with renal failure. In the event of convulsions, intravenous diazepam should be administered cautiously. Besides discontinuing treatment, unscheduled haemodialysis might be considered as a treatment alternative in patients with severe baclofen toxicity. Haemodialysis effectively removes baclofen from the body, alleviated clinical symptoms of overdose and shortens the recovery time in these patients.

PACK SIZES

Aluminum-PVC blister strip of 10 tablets per strip. Box of 5, 10, 20 & 50 strips.

STORAGE CONDITIONS & PHARMACEUTICAL PRECAUTIONS

Do not store above 30°C.
Protect from moisture and light.
Keep out of reach of children.
Jauhi daripada kanak-kanak.
SHELF LIFE: Please refer to the outer box label.

Manufactured by:

SRIPRASIT PHARMA CO. LTD.
216 Moo 6, Suanluang, Krathum Baen,
Samut Sakhon 74110, Thailand.

DATE OF REVISION: March 2019

PIL0007 Rev:2

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